

MAT Program Readiness Checklist – Tennessee Primary Care Clinics

Leadership & Legal Authority

- ☐ Designate MAT clinical lead
- ☐ Confirm prescriber authority per TN Code §53-11-311
- ☐ DEA registration with Schedule III authority
- ☐ Ensure compliance with MAT Act (no X-waiver, DEA training met)
- ☐ Determine eligibility/requirements for NP or PA prescribing (BESMART/OBOT if applicable)

State Regulatory & Policy Requirements

- ☐ Review TN Department of Health & TDMHSAS buprenorphine guidelines
- ☐ Establish MAT policies & procedures
- ☐ Create SUD informed consent & treatment contract
- ☐ Ensure alignment with 42 CFR Part 2 & HIPAA

PDMP (CSMD) & Monitoring Infrastructure

- ☐ Access to Tennessee Controlled Substance Monitoring Database (CSMD)
- ☐ PDMP review workflow defined (baseline & every visit)
- ☐ Documentation standard for PDMP review

Urine Drug Screening & Safety Protocols

- ☐ Standing urine drug screen protocol established
- ☐ TN-compliant UDS frequency policy documented
- ☐ Observation, chain-of-custody process defined
- ☐ Clear response pathway for unexpected results

EHR Build & Documentation

- ☐ MAT intake & follow-up templates built
- ☐ Standing orders for UDS & labs
- ☐ Smart phrases for PDMP, UDS, counseling
- ☐ Secure documentation practices in place

Clinic Operations & Staffing

- ☐ MAT scheduling templates created
- ☐ Staff trained in stigma- and trauma-informed care
- ☐ Defined roles for nursing, pill counts, coordination
- ☐ No-show & early refill policies finalized

Pharmacy & Medication Controls

- ☐ Single pharmacy policy established
- ☐ Pill/film count workflow defined
- ☐ Naloxone prescribing & education workflow in place

Behavioral Health & Community Linkage

- ☐ Therapy and psychiatry referral network identified
- ☐ NA/SMART Recovery resource list available
- ☐ Care coordination process documented

Quality & Risk Management

- ☐ Relapse & diversion response protocols written
- ☐ MAT quality metrics identified
- ☐ Transfer/termination of care workflow defined (non-punitive)